

nAMD

First Line Therapy

Aflibercept 2mg Biosimilar (Alternative: Ranibizumab)

Loading Phase: 3 injections every 4 weeks, OCT at 3rd visit. 4th injection at 8 weeks with OCT, then T&E* until 16 weeks (x2) **

Second Line Therapy – Loading NOT mandatory when switching from 1st to 2nd line - Clinician discretion

Aflibercept 8mg (HD Eylea)

Loading dose: 3 injections every 4 weeks, OCT at 3rd visit. 4th injection at 8 weeks with OCT, then T&E* until 16 weeks** (x2) No minimum dose interval

OR

Faricimab (Vabysmo)

Loading dose: 3 injections every 4 weeks, OCT at 3rd visit. 4th injection at 8 weeks with OCT, then T&E* until 16 weeks** (x2)

Third Line Therapy

Bevacizumab (Avastin)

Loading dose: 3 injections every 4 weeks, OCT at 3rd visit. 4th injection at 8 weeks with OCT, then T&E* until 16 weeks** (x2)

Patients need to be seen at least every 6 months in MR clinic

*If Anti-VEGF cannot be extended beyond Q8W interval consider switching

**End of pathway protocol for nAMD: MR clinic 4 months after last injection

NICE TA Criteria

- BCVA between 6/12 and 6/96
 - No permanent structural damage to the central fovea
 - Lesion size $\leq$ 12-disc areas in greatest linear dimension
 - Evidence of recent disease progression
- If outside, then **Bevacizumab can be offered**

Suboptimal response - Continue with REGULAR intervals: If active disease remains, maintain regular dosing. Switch to a second-choice option if active disease persists 4–8 weeks after the last injection.

If features of persistent active disease, such as **persistent haemorrhage**, subretinal fluid, or SRHM, treatment intervals should be **reduced**. A **4-weekly interval** should be maintained if the disease is known to worsen with longer intervals

Consider STOPPING

VA <15 letters attributed to nAMD on 2 consecutive visits and not the better seeing eye
OR
Irreversible structural changes WITH no prospect of visual improvement with continued treatment

DMO

First Line Therapy

Maximum switches: 4

Aflibercept 2mg Biosimilar (Alternative: Ranibizumab) – Ozurdex can be offered as 1st line if appropriate

Loading Phase: **4 injections every 4 weeks, OCT at 4th visit.** 5th injection at 8 weeks with OCT, then T&E* until 16 weeks**

Response Assessment at Week 24 (5th injection visit)
Good Response: $\geq 20\%$ reduction in CRT OR ≥ 5 letter gain in VA. → Continue T&E
Suboptimal Response: $<20\%$ reduction in CRT AND < 5 letter gain in VA → Consider Switch
Poor Response: Increase in CRT OR decrease in VA. → Switch

Consider STOPPING

VA <15 letters attributed to DMO
OR
No response (i.e. no change or worsening CRT AND VA)
OR
Irreversible structural changes WITH no prospect of visual improvement with continued treatment

Second Line Therapy - Loading NOT mandatory when switching from 1st to 2nd line – Clinician discretion

Aflibercept 8mg (HD Eylea)

Loading dose: 3 injections every 4 weeks, OCT at 3rd visit. 4th injection at 8 weeks with OCT, then T&E* until 16 weeks**

OR

Faricimab (Vabysmo)

Loading dose: 3 injections every 4 weeks, OCT at 3rd visit. 4th injection at 8 weeks with OCT, then T&E* until 16 weeks**

OR

Dexamethasone Implant (Ozurdex) - More appropriate (Recent CV event – within 6 months/Pregnancy/Unable to comply with frequent injections)

Can be repeated 6 monthly (max 7 per eye). Phakic/Pseudophakic. If responsive, can be considered for **Fluocinolone acetonide (Iluvien) implant (2 implants total per eye - repeat after 12 months if worsening DMO or after at least 3 years (if responded to initial implant))**

Patients need to be seen at least every 6 months in MR clinic

*If Anti-VEGF cannot be extended beyond Q8W interval consider switching

**End of pathway protocol for DMO: MR clinic 4 months after last injection

T&E: 2-4 weeks increment

v.8.4 updated on 29.04.26

RVO

First Line Therapy

Maximum switches: 3

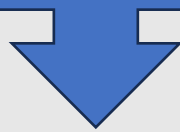
Aflibercept 2mg Biosimilar (Alternative: Ranibizumab) – Ozurdex can be offered as 1st line if appropriate

Loading Phase: 3 injections every 4 weeks, OCT at 3rd visit. 4th injection at 8 weeks with OCT, then T&E* until 16 weeks**

Favourable Disease Outcome (Resolution of CI-MO) - Continue T&E

Unfavourable Disease Outcome (Reduced but persistent CI-MO) – Switch to Faricimab OR Ozurdex OR Continue up to 3 more monthly injections and reassess at 6 months

Poor Disease Outcome (No change/worsening in VA and CMT) - Switch to Faricimab OR Ozurdex OR Stop



Consider STOPPING

VA <15 letters attributed to RVO
OR
No response (i.e. no change or worsening CI-MO compared to baseline)
OR
Complete resolution of CI-MO with no potential for visual acuity improvement

Second Line Therapy - Loading NOT mandatory when switching from 1st to 2nd line – Clinician discretion

Faricimab (Vabysmo)

Loading dose: 3 injections every 4 weeks, OCT at 3rd visit. 4th injection at 8 weeks with OCT, then T&E* until 16 weeks**

OR

Dexamethasone Implant (Ozurdex) - *More appropriate (Recent CV event – within 6 months/Pregnancy/Unable to comply with frequent injections)*

Can be repeated 6 monthly (**Maximum 6 implants per eye**). Phakic/Pseudophakic. IOP monitoring mandatory

Patients need to be seen at least every 6 months in MR clinic

*If Anti-VEGF cannot be extended beyond Q8W interval consider switching

**End of pathway protocol for RVO: MR clinic 4 months after last injection